Ponatinib / Asciminib nella LMC – Newsletter interna (ultimi 15 giorni)

Generata il 01/11/2026

Generata il: 2026-11-01

Newsletter interna – sintesi operativa

Messaggi chiave (da leggere in 60 secondi):

- • Nel periodo emergono soprattutto temi su **Resistenza / T315I / mutazioni** (n=6).
- • Secondo filone: **Sequencing / linee di terapia** (n=5).
- • Terzo filone: **TFR / stop terapia** (n=5).

Cosa cambia per noi (azioni pratiche):

- • Rivedere/rafforzare **monitoraggio cardiovascolare e interazioni** (es. anticoagulanti) nei pazienti in TKI.
- • Valutare criteri interni per **consolidamento / de-escalation / TFR** nei pazienti in deep response.
- • Verificare percorso di **test mutazionale** e criteri di switch nelle resistenze (incl. T315I).

Safety radar (segnali/interazioni da monitorare):

- • Monitorare evoluzione su **TFR / stop terapia** (nuovi segnali/dati).
- • Monitorare evoluzione su **Safety e comorbidità** (nuovi segnali/dati).
- • Monitorare evoluzione su **Resistenza / T315I / mutazioni** (nuovi segnali/dati).

Top 5 da leggere subito (ordinati per impatto)

1) Treatment-free remission after bosutinib discontinuation (BOSUTFR): results from a multicenter real-world study by the Spanish CML group.

Real-world / osservazionale *Annals of hematology* *Score:* ***16***

***Focus clinico**: TFR / stop terapia, Sequencing / linee di terapia, Real-world (utilità pratica)*

PMID: 42955104 | 10.1007/s00277-026-07039-4

`Link PubMed: https://pubmed.ncbi.nlm.nih.gov/42955104/`

- • **Tipo di studio/popolazione:** Treatment-free remission (TFR) is an established therapeutic goal for patients with chronic myeloid leukemia (CML) achieving sustained deep molecular response (DMR) on tyrosine kinase inhibitors (TKIs).
- • **Risultato chiave:** BOSUTFR is a multicenter, retrospective study by the Spanish CML Group (GELMC) evaluating TFR feasibility after bosutinib cessation in patients with chronic-phase CML.
- • **Rilevanza clinica:** All patients with molecular recurrence regained DMR after restarting bosutinib (n = 6) or switching to asciminib (n = 2).

2) [Treatment strategies for chronic-phase chronic myeloid leukemia in 2026].

Review *[Rinsho ketsueki] The Japanese journal of clinical hematology* Score: ***11***

***Focus clinico**: Resistenza / T315I / mutazioni, TFR / stop terapia, Dose strategy / consolidamento, Safety e comorbidità, Sequencing / linee di terapia, Sintesi (review/linee guida)*

PMID: 42951631 | 10.11406/rinketsu.67.1159

`Link PubMed: https://pubmed.ncbi.nlm.nih.gov/42951631/`

- • **Tipo di studio/popolazione:** The prognosis of chronic-phase chronic myeloid leukemia (CP-CML) has improved substantially since the introduction of tyrosine kinase inhibitor (TKI) therapy.
- • **Risultato chiave:** This review discusses first-line treatment selection based on the EUTOS long-term survival (ELTS) score, patient age, and the risk of vascular adverse events; the efficacy and safety profiles of second-generation TKIs, ponatinib, and asciminib; molecular response milestones according to treatment goals; and current approaches to TKI dose optimization.
- • **Rilevanza clinica:** Furthermore, it summarizes treatment selection in the second-line and later settings based on BCR::ABL1 mutation analysis in patients with resistant disease, as well as the results of TFR clinical trials conducted in Japan.

3) Twenty-Five Years of CML Therapy in India: A Model for Precision Oncology in a Resource-Constrained Setting.

Review *Clinical lymphoma, myeloma & leukemia* Score: ***10***

***Focus clinico**: Resistenza / T315I / mutazioni, TFR / stop terapia, Dose strategy / consolidamento, Real-world (utilità pratica), Sintesi (review/linee guida)*

PMID: 42907307 | 10.1016/j.clml.2026.09.021

`Link PubMed: https://pubmed.ncbi.nlm.nih.gov/42907307/`

- • **Tipo di studio/popolazione:** Chronic myeloid leukemia (CML) is the classic example in oncology of a single molecular lesion turned into a manageable, and increasingly curable, disease.
- • **Risultato chiave:** The disease presents in India roughly 2 decades earlier than in Western registries, more often symptomatically and at a higher risk score, yet almost always still in chronic phase, and it is treated, overwhelmingly, with generic imatinib.
- • **Rilevanza clinica:** Several Indian contributions are highlighted: generic imatinib reproduces the innovator's response, survival and treatment-free-remission outcomes at a fraction of the cost, sustained BCR::ABL1 below 1% is a legitimate goal, and deeper responses, while desirable, are not mandatory, low-dose (50 mg) dasatinib achieves deep responses while sparing the pleural and pericardial toxicity seen at standard doses, allogeneic transplantation remains a one-time, cost-effective cure for T315I and multi-TKI-failure disease, far cheaper than ponatinib or asciminib and a young patient population has generated some of the world's largest CML-in-pregnancy series, with reassuring outcomes.

4) Asciminib After Allogeneic Hematopoietic Stem Cell Transplantation in Chronic Myeloid Leukemia: A Multicenter Real-World Experience.

Real-world / osservazionale *Clinical lymphoma, myeloma & leukemia* Score: ***9***

***Focus clinico**: Resistenza / T315I / mutazioni, Dose strategy / consolidamento, Safety e comorbidità, Real-world (utilità pratica)*

PMID: 42926184 | 10.1016/j.clml.2026.09.020

`Link PubMed: https://pubmed.ncbi.nlm.nih.gov/42926184/`

- • **Tipo di studio/popolazione:** Asciminib, a first-in-class STAMP inhibitor, has demonstrated efficacy in chronic-phase chronic myeloid leukemia (CML) after failure of prior TKIs (tyrosine kinase inhibitor), including T315I-mutated disease.
- • **Risultato chiave:** We conducted a multicenter retrospective analysis of 22 patients with CML who received asciminib after allo-HSCT performed between August 2021 and November 2025.
- • **Rilevanza clinica:** Low-dose asciminib (20-80 mg daily) was generally feasible and well tolerated after allogeneic HSCT, including in patients with the T315I mutation and graft hypofunction, and MR5 was achieved in most patients.

5) Single-Nucleus Transcriptomics Reveal Mechanisms of Cardiovascular Protection Against Ponatinib-Induced Cardiotoxicity Through ROCKs Deletion in Cardiomyocytes.

Preclinico / laboratorio *Archivum immunologiae et therapiae experimentalis* Score: ***8***

***Focus clinico**: Resistenza / T315I / mutazioni, Safety e comorbidità, Sequencing / linee di terapia*

PMID: 42963886 | 10.2478/aite-2026-0027

`Link PubMed: https://pubmed.ncbi.nlm.nih.gov/42963886/`

- • **Tipo di studio/popolazione:** Ponatinib is a potent tyrosine kinase inhibitor and primarily used in the treatment of chronic myeloid leukemia (CML) and Philadelphia chromosome-positive acute lymphoblastic leukemia (Ph + ALL).
- • **Risultato chiave:** Its broad anti-cancer capability is restricted to CML or Ph + ALL harboring T315I mutant breakpoint cluster region gene fused with the Abelson murine leukemia gene 1 due to its potential cardiotoxicity (CTX) side effect.
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Evidenze per domande cliniche

Resistenza / T315I / mutazioni (6)

Score: ***0***

Mini-riassunto (da abstract, estrattivo):

[Treatment strategies for chronic-phase chronic myeloid leukemia in 2026].

Review *[Rinsho ketsueki] The Japanese journal of clinical hematology* Score: ***11***

PMID: 42951631 | 10.11406/rinketsu.67.1159

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Open Access (Europe PMC/PMC): non rilevato

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Mini-riassunto (da abstract, estrattivo):

Twenty-Five Years of CML Therapy in India: A Model for Precision Oncology in a Resource-Constrained Setting.

Review *Clinical lymphoma, myeloma & leukemia* Score: ***10***

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Preclinico / laboratorio *Archivum immunologiae et therapiae experimentalis* Score: ***8***

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- • **Rilevanza clinica:** We first assessed the role of Rho-associated coiled-coil containing kinases 1 and 2 (ROCK1 and ROCK2) in mediating CTX on mouse models characterized by the inducible cardiomyocyte-specific double ROCK knockout (DKO).

Mini-riassunto (da abstract, estrattivo):

Riassunto aggiuntivo (full text Open Access, estrattivo):

Exploring the potential of vamotinib in Ph+ leukemias: is it indeed a 3rd generation tyrosine kinase-inhibitor?

Preclinico / laboratorio *Journal of hematology & oncology* *Score:* ***7***

PMID: 42934771 | 10.1186/s13045-026-01835-1

`Link PubMed: https://pubmed.ncbi.nlm.nih.gov/42934771/`

Open Access (Europe PMC/PMC): sì (PMC13700276)

- • **Tipo di studio/popolazione:** While the third-generation TKIs ponatinib and asciminib can suppress many resistant mutations including the gatekeeper mutation T315I, their use may be limited by vascular toxicity and emerging resistance, respectively.
- • **Risultato chiave:** Compound mutations (CMs), particularly those involving T315I, are frequent in advanced chronic myeloid leukemia (CML) and Philadelphia chromosome-positive acute lymphoblastic leukemia and are largely refractory to approved TKIs when used as monotherapy.
- • **Rilevanza clinica:** Clinical responses in mutation-positive patients were largely concordant with these observations.
- • **Metodi (full text OA):** non riportato.
- • **Risultato principale (full text OA):** non riportato.
- • **Dettaglio utile (full text OA):** non riportato.
- • **Messaggio clinico (full text OA):** non riportato.

Mini-riassunto (da abstract, estrattivo):

Current perspectives on high-risk genomic profiles in chronic myeloid leukemia: from bench to clinical practice.

Preclinico / laboratorio *International journal of hematology* *Score:* ***2***

PMID: 42905187 | 10.1007/s12185-026-04301-3

`Link PubMed: https://pubmed.ncbi.nlm.nih.gov/42905187/`

Open Access (Europe PMC/PMC): non rilevato

- • **Tipo di studio/popolazione:** Chronic myeloid leukemia (CML) is defined by BCR::ABL1 rearrangement, yet its genomic architecture extends well beyond this single driver.
- • **Risultato chiave:** ASXL1 mutations are present in 7-14% of newly diagnosed CP patients and independently predict inferior response, treatment failure, and ABL1 kinase domain mutations, even with potent frontline tyrosine kinase inhibitors (TKIs), including asciminib.
- • **Rilevanza clinica:** A newly described sixth pattern of mutation dynamics-de novo DTA (DNMT3A, TET2, ASXL1) mutations in 24% of optimal responders with a median doubling time of 68 days-supports serial NGS monitoring every 6-7 months as a noninvasive complement to cytogenetics.

Sequencing / linee di terapia (5)

Score: ***0***

Mini-riassunto (da abstract, estrattivo):

Riassunto aggiuntivo (full text Open Access, estrattivo):

Treatment-free remission after bosutinib discontinuation (BOSUTFR): results from a multicenter real-world study by the Spanish CML group.

Real-world / osservazionale *Annals of hematology* Score: ***16***

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- • **Rilevanza clinica:** All patients with molecular recurrence regained DMR after restarting bosutinib (n = 6) or switching to asciminib (n = 2).
- • **Metodi (full text OA):** Eligible patients were adults (≥ 18 years) with confirmed CML-CP who had received bosutinib for any line of therapy.
- • **Risultato principale (full text OA):** All patients who lost TFR after discontinuation regained at least a DMR after restarting bosutinib (n = 6) or switching to asciminib (n = 2).
- • **Dettaglio utile (full text OA):** those who discontinued due to a sustained molecular response ≥ 4.0 (blue line), according to ELN recommendations Clinical characteristics were similar between patients with and without molecular recurrence (Table 1).
- • **Messaggio clinico (full text OA):** This suggests that TFR may be attainable in a broader range of patients, including those with less stringent molecular responses than current ELN guidelines imply [1].

Mini-riassunto (da abstract, estrattivo):

[Treatment strategies for chronic-phase chronic myeloid leukemia in 2026].

Review *[Rinsho ketsueki] The Japanese journal of clinical hematology* Score: ***11***

PMID: 42951631 | 10.11406/rinketsu.67.1159

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Real-world outcomes of asciminib versus first- and second-generation tyrosine kinase inhibitors as second-line treatment for chronic-phase chronic myeloid leukemia.

Review *Leukemia research* Score: ***6***

PMID: 42913224 | 10.1016/j.leukres.2026.108345

`Link PubMed: https://pubmed.ncbi.nlm.nih.gov/42913224/`

Open Access (Europe PMC/PMC): non rilevato

- • **Tipo di studio/popolazione:** This study compared real-world outcomes of patients with chronic myeloid leukemia in chronic phase (CML-CP) treated with asciminib and first/second generation (1 G/2 G) tyrosine kinase inhibitors (TKI) as second-line treatment (2 L) in the US.
- • **Risultato chiave:** A retrospective panel-based chart review from oncologists/hematologists was conducted on adult patients with CML-CP in 2 L treated with asciminib or 1 G/2 G TKIs between January 2022-June 2023 (index period).
- • **Rilevanza clinica:** Findings from this large real-world US study suggest that 2 L asciminib offers favorable tolerability and better effectiveness compared with 1 G/2 G TKIs for patients with CML-CP.

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Current perspectives on high-risk genomic profiles in chronic myeloid leukemia: from bench to clinical practice.

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TFR / stop terapia (5)

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Mini-riassunto (da abstract, estrattivo):

Riassunto aggiuntivo (full text Open Access, estrattivo):

Dasatinib-Associated Gastrointestinal Bleeding and Colitis: A Narrative Review with Illustrative Cases.

Review *Journal of clinical medicine* *Score:* ***4***

PMID: 42941598 | 10.3390/jcm15208086

`Link PubMed: https://pubmed.ncbi.nlm.nih.gov/42941598/`

Open Access (Europe PMC/PMC): sì (PMC13706206)

- • **Tipo di studio/popolazione:** Dasatinib-associated colitis is far more extensively reported than non-colitis bleeding, has been reported across a broad adult age range and in both sexes, and has onset ranging from immediate to 9 years after initiation.
- • **Risultato chiave:** The review is illustrated by two cases from a tertiary referral centre: delayed bleeding after cold-snare polypectomy of a diminutive 3 mm colorectal polyp, and chronic active colitis on combination asciminib-dasatinib therapy; formal causality assessment rated a dasatinib contribution as possible and probable, respectively.
- • **Rilevanza clinica:** Standardised diagnostic criteria and peri-procedural guidance are needed to better balance bleeding risk against leukaemia control.
- • **Metodi (full text OA):** The probability that dasatinib contributed to each event was assessed using the Naranjo adverse drug reaction probability scale [19] and the WHO–Uppsala Monitoring Centre (WHO-UMC) causality categories [20].
- • **Risultato principale (full text OA):** non riportato.
- • **Dettaglio utile (full text OA):** non riportato.
- • **Messaggio clinico (full text OA):** Initial treatment typically involves temporary cessation with corticosteroids in selected cases; three published cases continued or resumed dasatinib at a lower dose with reported clinical improvement [13,15,34], and the institutional Case 2 described in this review similarly showed symptomatic improvement after sequential dose reduction, though without a documented complete response, and dose reduction to 50 mg daily maintains efficacy with a more favourable overall GI safety profile in newly diagnosed CML-CP [73]—though this frontline population and endpoint differ from colitis-specific dose reduction in previously treated patients—although a clear dose–response relationship for colitis itself has not been established.

Real-world (utilità pratica) (5)

Score: ***0***

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Riassunto aggiuntivo (full text Open Access, estrattivo):

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`Link PubMed: https://pubmed.ncbi.nlm.nih.gov/42955104/`

Open Access (Europe PMC/PMC): sì (PMC13719861)

- • **Tipo di studio/popolazione:** Treatment-free remission (TFR) is an established therapeutic goal for patients with chronic myeloid leukemia (CML) achieving sustained deep molecular response (DMR) on tyrosine kinase inhibitors (TKIs).
- • **Risultato chiave:** BOSUTFR is a multicenter, retrospective study by the Spanish CML Group (GELMC) evaluating TFR feasibility after bosutinib cessation in patients with chronic-phase CML.
- • **Rilevanza clinica:** All patients with molecular recurrence regained DMR after restarting bosutinib (n = 6) or switching to asciminib (n = 2).
- • **Metodi (full text OA):** Eligible patients were adults (≥ 18 years) with confirmed CML-CP who had received bosutinib for any line of therapy.
- • **Risultato principale (full text OA):** All patients who lost TFR after discontinuation regained at least a DMR after restarting bosutinib (n = 6) or switching to asciminib (n = 2).
- • **Dettaglio utile (full text OA):** those who discontinued due to a sustained molecular response ≥ 4.0 (blue line), according to ELN recommendations Clinical characteristics were similar between patients with and without molecular recurrence (Table 1).
- • **Messaggio clinico (full text OA):** This suggests that TFR may be attainable in a broader range of patients, including those with less stringent molecular responses than current ELN guidelines imply [1].

Mini-riassunto (da abstract, estrattivo):

Twenty-Five Years of CML Therapy in India: A Model for Precision Oncology in a Resource-Constrained Setting.

Review *Clinical lymphoma, myeloma & leukemia* *Score:* ***10***

PMID: 42907307 | 10.1016/j.clml.2026.09.021

`Link PubMed: https://pubmed.ncbi.nlm.nih.gov/42907307/`

Open Access (Europe PMC/PMC): non rilevato

- • **Tipo di studio/popolazione:** Chronic myeloid leukemia (CML) is the classic example in oncology of a single molecular lesion turned into a manageable, and increasingly curable, disease.
- • **Risultato chiave:** The disease presents in India roughly 2 decades earlier than in Western registries, more often symptomatically and at a higher risk score, yet almost always still in chronic phase, and it is treated, overwhelmingly, with generic imatinib.
- • **Rilevanza clinica:** Several Indian contributions are highlighted: generic imatinib reproduces the innovator's response, survival and treatment-free-remission outcomes at a fraction of the cost, sustained BCR::ABL1 below 1% is a legitimate goal, and deeper responses, while desirable, are not mandatory, low-dose (50 mg) dasatinib achieves deep responses while sparing the pleural and pericardial toxicity seen at standard doses, allogeneic transplantation remains a one-time, cost-effective cure for T315I and multi-TKI-failure disease, far cheaper than ponatinib or asciminib and a young patient population has generated some of the world's largest CML-in-pregnancy series, with reassuring outcomes.

Mini-riassunto (da abstract, estrattivo):

Asciminib After Allogeneic Hematopoietic Stem Cell Transplantation in Chronic Myeloid Leukemia: A Multicenter Real-World Experience.

Real-world / osservazionale *Clinical lymphoma, myeloma & leukemia* *Score:* ***9***

PMID: 42926184 | 10.1016/j.clml.2026.09.020

`Link PubMed: https://pubmed.ncbi.nlm.nih.gov/42926184/`

Open Access (Europe PMC/PMC): non rilevato

- • **Tipo di studio/popolazione:** Asciminib, a first-in-class STAMP inhibitor, has demonstrated efficacy in chronic-phase chronic myeloid leukemia (CML) after failure of prior TKIs (tyrosine kinase inhibitor), including T315I-mutated disease.
- • **Risultato chiave:** We conducted a multicenter retrospective analysis of 22 patients with CML who received asciminib after allo-HSCT performed between August 2021 and November 2025.
- • **Rilevanza clinica:** Low-dose asciminib (20-80 mg daily) was generally feasible and well tolerated after allogeneic HSCT, including in patients with the T315I mutation and graft hypofunction, and MR5 was achieved in most patients.

Mini-riassunto (da abstract, estrattivo):

Riassunto aggiuntivo (full text Open Access, estrattivo):

Correction to: Real-world outcomes of ponatinib in heavily pretreated patients with chronic myeloid leukemia and Philadelphia chromosome-positive acute lymphoblastic leukemia.

Real-world / osservazionale *Annals of hematology* *Score:* ***6***

PMID: 42925414 | 10.1007/s00277-026-07311-7

`Link PubMed: https://pubmed.ncbi.nlm.nih.gov/42925414/`

Open Access (Europe PMC/PMC): sì (PMC13695665)

- • **Tipo di studio/popolazione:** non riportato (abstract non disponibile).
- • **Risultato chiave:** non riportato.
- • **Rilevanza clinica:** non valutabile senza abstract.
- • **Metodi (full text OA):** non riportato.
- • **Risultato principale (full text OA):** non riportato.
- • **Dettaglio utile (full text OA):** non riportato.
- • **Messaggio clinico (full text OA):** non riportato.

Mini-riassunto (da abstract, estrattivo):

Real-world outcomes of asciminib versus first- and second-generation tyrosine kinase inhibitors as second-line treatment for chronic-phase chronic myeloid leukemia.

Review *Leukemia research* Score: ***6***

PMID: 42913224 | 10.1016/j.leukres.2026.108345

`Link PubMed: https://pubmed.ncbi.nlm.nih.gov/42913224/`

Open Access (Europe PMC/PMC): non rilevato

- • **Tipo di studio/popolazione:** This study compared real-world outcomes of patients with chronic myeloid leukemia in chronic phase (CML-CP) treated with asciminib and first/second generation (1 G/2 G) tyrosine kinase inhibitors (TKI) as second-line treatment (2 L) in the US.
- • **Risultato chiave:** A retrospective panel-based chart review from oncologists/hematologists was conducted on adult patients with CML-CP in 2 L treated with asciminib or 1 G/2 G TKIs between January 2022-June 2023 (index period).
- • **Rilevanza clinica:** Findings from this large real-world US study suggest that 2 L asciminib offers favorable tolerability and better effectiveness compared with 1 G/2 G TKIs for patients with CML-CP.

Safety e comorbidità (4)

Score: ***0***

Mini-riassunto (da abstract, estrattivo):

[Treatment strategies for chronic-phase chronic myeloid leukemia in 2026].

Review *[Rinsho ketsueki] The Japanese journal of clinical hematology* Score: ***11***

PMID: 42951631 | 10.11406/rinketsu.67.1159

`Link PubMed: https://pubmed.ncbi.nlm.nih.gov/42951631/`

Open Access (Europe PMC/PMC): non rilevato

- • **Tipo di studio/popolazione:** The prognosis of chronic-phase chronic myeloid leukemia (CP-CML) has improved substantially since the introduction of tyrosine kinase inhibitor (TKI) therapy.
- • **Risultato chiave:** This review discusses first-line treatment selection based on the EUTOS long-term survival (ELTS) score, patient age, and the risk of vascular adverse events; the efficacy and safety profiles of second-generation TKIs, ponatinib, and asciminib; molecular response milestones according to treatment goals; and current approaches to TKI dose optimization.
- • **Rilevanza clinica:** Furthermore, it summarizes treatment selection in the second-line and later settings based on BCR::ABL1 mutation analysis in patients with resistant disease, as well as the results of TFR clinical trials conducted in Japan.

Mini-riassunto (da abstract, estrattivo):

Asciminib After Allogeneic Hematopoietic Stem Cell Transplantation in Chronic Myeloid Leukemia: A Multicenter Real-World Experience.

Real-world / osservazionale *Clinical lymphoma, myeloma & leukemia* Score: ***9***

PMID: 42926184 | 10.1016/j.clml.2026.09.020

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- • **Tipo di studio/popolazione:** Asciminib, a first-in-class STAMP inhibitor, has demonstrated efficacy in chronic-phase chronic myeloid leukemia (CML) after failure of prior TKIs (tyrosine kinase inhibitor), including T315I-mutated disease.
- • **Risultato chiave:** We conducted a multicenter retrospective analysis of 22 patients with CML who received asciminib after allo-HSCT performed between August 2021 and November 2025.
- • **Rilevanza clinica:** Low-dose asciminib (20-80 mg daily) was generally feasible and well tolerated after allogeneic HSCT, including in patients with the T315I mutation and graft hypofunction, and MR5 was achieved in most patients.

Mini-riassunto (da abstract, estrattivo):

Single-Nucleus Transcriptomics Reveal Mechanisms of Cardiovascular Protection Against Ponatinib-Induced Cardiotoxicity Through ROCKs Deletion in Cardiomyocytes.

Preclinico / laboratorio *Archivum immunologiae et therapiae experimentalis* Score: ***8***

PMID: 42963886 | 10.2478/aite-2026-0027

`Link PubMed: https://pubmed.ncbi.nlm.nih.gov/42963886/`

Open Access (Europe PMC/PMC): non rilevato

- • **Tipo di studio/popolazione:** Ponatinib is a potent tyrosine kinase inhibitor and primarily used in the treatment of chronic myeloid leukemia (CML) and Philadelphia chromosome-positive acute lymphoblastic leukemia (Ph + ALL).
- • **Risultato chiave:** Its broad anti-cancer capability is restricted to CML or Ph + ALL harboring T315I mutant breakpoint cluster region gene fused with the Abelson murine leukemia gene 1 due to its potential cardiotoxicity (CTX) side effect.
- • **Rilevanza clinica:** We first assessed the role of Rho-associated coiled-coil containing kinases 1 and 2 (ROCK1 and ROCK2) in mediating CTX on mouse models characterized by the inducible cardiomyocyte-specific double ROCK knockout (DKO).

Mini-riassunto (da abstract, estrattivo):

Riassunto aggiuntivo (full text Open Access, estrattivo):

Exploring the potential of vamotinib in Ph+ leukemias: is it indeed a 3rd generation tyrosine kinase-inhibitor?

Preclinico / laboratorio *Journal of hematology & oncology* *Score: **7***

PMID: 42934771 | 10.1186/s13045-026-01835-1

`Link PubMed: https://pubmed.ncbi.nlm.nih.gov/42934771/`

Open Access (Europe PMC/PMC): sì (PMC13700276)

- • **Tipo di studio/popolazione:** While the third-generation TKIs ponatinib and asciminib can suppress many resistant mutations including the gatekeeper mutation T315I, their use may be limited by vascular toxicity and emerging resistance, respectively.
- • **Risultato chiave:** Compound mutations (CMs), particularly those involving T315I, are frequent in advanced chronic myeloid leukemia (CML) and Philadelphia chromosome-positive acute lymphoblastic leukemia and are largely refractory to approved TKIs when used as monotherapy.
- • **Rilevanza clinica:** Clinical responses in mutation-positive patients were largely concordant with these observations.
- • **Metodi (full text OA):** non riportato.
- • **Risultato principale (full text OA):** non riportato.
- • **Dettaglio utile (full text OA):** non riportato.
- • **Messaggio clinico (full text OA):** non riportato.

Dose strategy / consolidamento (4)

*Score: **0***

Mini-riassunto (da abstract, estrattivo):

[Treatment strategies for chronic-phase chronic myeloid leukemia in 2026].

Review *[Rinsho ketsueki] The Japanese journal of clinical hematology* *Score: **11***

PMID: 42951631 | 10.11406/rinketsu.67.1159

`Link PubMed: https://pubmed.ncbi.nlm.nih.gov/42951631/`

Open Access (Europe PMC/PMC): non rilevato

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- • **Rilevanza clinica:** Furthermore, it summarizes treatment selection in the second-line and later settings based on BCR::ABL1 mutation analysis in patients with resistant disease, as well as the results of TFR clinical trials conducted in Japan.

Mini-riassunto (da abstract, estrattivo):

Twenty-Five Years of CML Therapy in India: A Model for Precision Oncology in a Resource-Constrained Setting.

Review *Clinical lymphoma, myeloma & leukemia* *Score:* ***10***

PMID: 42907307 | 10.1016/j.clml.2026.09.021

`Link PubMed: https://pubmed.ncbi.nlm.nih.gov/42907307/`

Open Access (Europe PMC/PMC): non rilevato

- • **Tipo di studio/popolazione:** Chronic myeloid leukemia (CML) is the classic example in oncology of a single molecular lesion turned into a manageable, and increasingly curable, disease.
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Asciminib After Allogeneic Hematopoietic Stem Cell Transplantation in Chronic Myeloid Leukemia: A Multicenter Real-World Experience.

Real-world / osservazionale *Clinical lymphoma, myeloma & leukemia* *Score:* ***9***

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Preclinico / laboratorio *Journal of hematology & oncology* *Score: **7***

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- • **Rilevanza clinica:** Clinical responses in mutation-positive patients were largely concordant with these observations.
- • **Metodi (full text OA):** non riportato.
- • **Risultato principale (full text OA):** non riportato.
- • **Dettaglio utile (full text OA):** non riportato.
- • **Messaggio clinico (full text OA):** non riportato.

Sintesi (review/linee guida) (4)

*Score: **0***

Mini-riassunto (da abstract, estrattivo):

[Treatment strategies for chronic-phase chronic myeloid leukemia in 2026].

Review *[Rinsho ketsueki] The Japanese journal of clinical hematology* *Score: **11***

PMID: 42951631 | 10.11406/rinketsu.67.1159

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Review *Clinical lymphoma, myeloma & leukemia* *Score:* ***10***

PMID: 42907307 | 10.1016/j.clml.2026.09.021

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- • **Risultato chiave:** The disease presents in India roughly 2 decades earlier than in Western registries, more often symptomatically and at a higher risk score, yet almost always still in chronic phase, and it is treated, overwhelmingly, with generic imatinib.
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Mini-riassunto (da abstract, estrattivo):

Real-world outcomes of asciminib versus first- and second-generation tyrosine kinase inhibitors as second-line treatment for chronic-phase chronic myeloid leukemia.

Review *Leukemia research* *Score:* ***6***

PMID: 42913224 | 10.1016/j.leukres.2026.108345

`Link PubMed: https://pubmed.ncbi.nlm.nih.gov/42913224/`

Open Access (Europe PMC/PMC): non rilevato

- • **Tipo di studio/popolazione:** This study compared real-world outcomes of patients with chronic myeloid leukemia in chronic phase (CML-CP) treated with asciminib and first/second generation (1 G/2 G) tyrosine kinase inhibitors (TKI) as second-line treatment (2 L) in the US.
- • **Risultato chiave:** A retrospective panel-based chart review from oncologists/hematologists was conducted on adult patients with CML-CP in 2 L treated with asciminib or 1 G/2 G TKIs between January 2022-June 2023 (index period).
- • **Rilevanza clinica:** Findings from this large real-world US study suggest that 2 L asciminib offers favorable tolerability and better effectiveness compared with 1 G/2 G TKIs for patients with CML-CP.

Mini-riassunto (da abstract, estrattivo):

Riassunto aggiuntivo (full text Open Access, estrattivo):

Dasatinib-Associated Gastrointestinal Bleeding and Colitis: A Narrative Review with Illustrative Cases.

Review *Journal of clinical medicine* *Score:* ***4***

PMID: 42941598 | 10.3390/jcm15208086

`Link PubMed: https://pubmed.ncbi.nlm.nih.gov/42941598/`

Open Access (Europe PMC/PMC): sì (PMC13706206)

- • **Tipo di studio/popolazione:** Dasatinib-associated colitis is far more extensively reported than non-colitis bleeding, has been reported across a broad adult age range and in both sexes, and has onset ranging from immediate to 9 years after initiation.
- • **Risultato chiave:** The review is illustrated by two cases from a tertiary referral centre: delayed bleeding after cold-snare polypectomy of a diminutive 3 mm colorectal polyp, and chronic active colitis on combination asciminib-dasatinib therapy; formal causality assessment rated a dasatinib contribution as possible and probable, respectively.
- • **Rilevanza clinica:** Standardised diagnostic criteria and peri-procedural guidance are needed to better balance bleeding risk against leukaemia control.
- • **Metodi (full text OA):** The probability that dasatinib contributed to each event was assessed using the Naranjo adverse drug reaction probability scale [19] and the WHO–Uppsala Monitoring Centre (WHO-UMC) causality categories [20].
- • **Risultato principale (full text OA):** non riportato.
- • **Dettaglio utile (full text OA):** non riportato.
- • **Messaggio clinico (full text OA):** Initial treatment typically involves temporary cessation with corticosteroids in selected cases; three published cases continued or resumed dasatinib at a lower dose with reported clinical improvement [13,15,34], and the institutional Case 2 described in this review similarly showed symptomatic improvement after sequential dose reduction, though without a documented complete response, and dose reduction to 50 mg daily maintains efficacy with a more favourable overall GI safety profile in newly diagnosed CML-CP [73]—though this frontline population and endpoint differ from colitis-specific dose reduction in previously treated patients—although a clear dose–response relationship for colitis itself has not been established.

Altro (1)

Score: ***0***

Mini-riassunto (da abstract, estrattivo):

Characterization of Adverse Cutaneous Effects in the Setting of Ponatinib, Bosutinib, and Asciminib for Chronic Myeloid Leukemia Patients.

Altro *Hematological oncology* *Score:* ***3***

PMID: 42899371 | 10.1002/hon.70280

`Link PubMed: https://pubmed.ncbi.nlm.nih.gov/42899371/`

Open Access (Europe PMC/PMC): non rilevato

- • **Tipo di studio/popolazione:** non riportato (abstract non disponibile).
- • **Risultato chiave:** non riportato.
- • **Rilevanza clinica:** non valutabile senza abstract.